

Contemporary issues

Challenging the narrative to reimagine health disparities research and education: Perspectives from international scholars

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ABSTRACT

This paper, drawing on the experiences of international and US-based nursing scholars, critically reflects on health disparities research and education in the US. We call for the need to expand health disparities discourse beyond race and ethnicity to include other axes of disparity, such as gender and sexuality, socioeconomic conditions, rural-urban divides, and displacement due to conflict. Examples from Ethiopia, Ghana, South Korea, Taiwan, and Rwanda illustrate how geopolitical factors, infrastructure, and local cultural contexts shape health disparities. We also critique the practice of binary and hierarchical racial comparisons, aggregation of racial and ethnic categories, and use of a single explanatory framework, all of which obscure important nuances and limit the scope of nursing research and education. We advocate culturally respectful approaches that apply intersectional lenses to each local context of health disparities. This process requires collaboration across multiple sectors, including researchers, clinicians, and community partners from diverse settings, in reciprocal learning environments. Our reflections offer actionable strategies and modalities for rethinking health disparities education. Globally nuanced perspectives, the ones examining multiple axes of disparities through collaboration of diverse sectors and backgrounds, can enrich nursing education, deepen understanding of how social determinants of health intersect in each context, and better equip future nursing researchers and practitioners to design and implement equitable healthcare interventions tailored to the needs of diverse populations.

1. Introduction

Addressing health disparities is a ubiquitous and foundational element of nursing pedagogy. In the United States (US), nursing education has increasingly addressed health disparities using the lens of race and racism—structural inequities and discrimination tied to racial categories (Lewis et al., 2022). Addressing structural injustice and racism is essential in the US, where social determinants like income, education, and housing are often racially stratified. Yet, educators should take care not to treat race as the sole axis of disparity in nursing classrooms (Blanchet Garneau et al., 2018), as this may inadvertently generalize US-based racial frameworks across global settings. Drawing on our experience as international scholars, we suggest complementary and alternative frameworks alongside race-based frameworks for application to diverse geopolitical and sociocultural contexts.

In US nursing curricula, addressing racial disparities and their root causes is crucial (Lewis et al., 2022). However, we posit in this paper that fully considering the socio-historical backgrounds and intersecting roles of sex, gender, and other social determinants beyond race alone is necessary to identify the underlying mechanisms driving local and global health disparities (Kimani, 2023). Our concerns particularly relate to the growing influence of US models in global education. For many international scholars from countries where nationality, religion, or class are more salient social identifiers, oversimplified race-based framings may seem unfamiliar (World Education Services, 2019). Students from racially homogeneous countries with limited exposure to racial oppression may hesitate to engage in classroom discussions for fear of making insensitive statements. At the same time, their experiences with colonial legacies, ethnic tensions, or authoritarian regimes offer insights into how disparities manifest beyond the US locally and

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highlight the need for culturally and contextually sensitive approaches to understanding health disparities, educating future nurses, and conducting research.

Celeste Chavez, a US-based scholar, reflects on her early encounters with international colleagues. She views international scholars as experts from their home countries who enrich the learning environment with new perspectives. To expand what we learn and teach in nursing classrooms, she calls for educational models that question US-centric assumptions and actively engage with global perspectives through reciprocal learning. Supporting this approach, [Nye and Dillard-Wright \(2023\)](#), drawing from [Kumashiro \(2015\)](#), emphasize the importance of critical self-reflection among nurse educators, urging them to interrogate their praxis by asking, ‘What are the hidden ways that my practices reinforce oppression?’

This paper offers a critical reflection on how nurse researchers and educators can engage with international scholars’ perspectives on health disparities. The authors—doctoral students, postdoctoral researchers, and early-career nurse scientists based at US educational institutions—draw from their experiences across Africa, Asia, and the US. Our positionalities in this collaborative reflection are shaped by assumptions, interpretations, and tensions from our home-country contexts ([Kohl and McCutcheon, 2015](#)). We recognize that the linguistic, financial, geopolitical, and institutional opportunities that enable us to participate in US academic settings influence which perspectives are represented in this paper. We are mindful of how inattentive treatment of certain disparities may unintentionally reinforce the imperial dynamics we aim to critique. This paper is one conversation across authors from different backgrounds, which is not representative of all international perspectives. Our perspectives do not dismiss the need to address race-based disparities or racism but call for more pluralistic, globally nuanced approaches—ones that examine multiple axes of disparity through collaboration among scholars, clinicians, and community members from diverse contexts.

2. Rethinking race-based disparities in research and education

Health disparities manifest differently across global contexts, influencing how they are conceptualized and taught in academic institutions ([Beckfield and Olafsdottir, 2013](#)) (See [Fig. 1](#)). In the US, discussions

often center on race and ethnicity embedded in the broader social determinants of health and call for legislative, policy, and structural interventions. Historic redlining shows how segregated housing policies restricted access to social and health services and produced long-term health impacts ([Lewis et al., 2022](#)). Thus, in this context, addressing the legacies of colonization and enduring harms of structural racism is necessary ([Lewis et al., 2022](#)). What we add is that, as the global influence of US-based scholarship and migration to the US continue to grow, nursing researchers and educators should further examine how power shapes what is taught, how it is framed, and whose knowledge is validated.

US epidemiological research commonly analyzes health disparities using binary racial comparisons between non-Hispanic White and Black groups ([Cartagena et al., 2022](#); [Kauh et al., 2021](#)). We raise caution when analyses overlook other significant factors—such as country of origin, citizenship, immigration status, and language proficiency—that shape the diverse identities of the US population, limiting opportunities to understand under-researched communities ([Kauh et al., 2021](#)).

To address this, disaggregating data of multiple, minoritized populations can critically help capture the unique needs and experiences of different ethnicities and heritages. Aggregating them into a single category or labeling “Others” can obscure meaningful within- and between-group variations ([Kauh et al., 2021](#)). To better incorporate those differences into tailored interventions, examining the specific historical, sociocultural, and economic contexts of each group can help reveal their unique contextual needs. Conducting more cross-cultural and cross-national studies can illuminate how different cultural norms and systemic factors shape population health ([Salway et al., 2011](#)).

The practice of using the non-Hispanic White group as the reference group in US epidemiologic research also reflects racialized assumptions positioning the non-Hispanic White group as the default comparator ([Flanagan et al., 2021](#)). For example, the observation that Hispanic women have maternal mortality outcomes similar to White women is called the “Latina Epidemiologic Paradox” ([Hoggatt et al., 2012](#)), which is a racializing framing that can obscure the underlying causes of how racial and ethnic disparities develop ([Cartagena et al., 2022](#)).

Biracial or multiracial individuals are also poorly represented or misclassified based on existing racial categories when research and education do not incorporate disaggregation into granular, intersecting

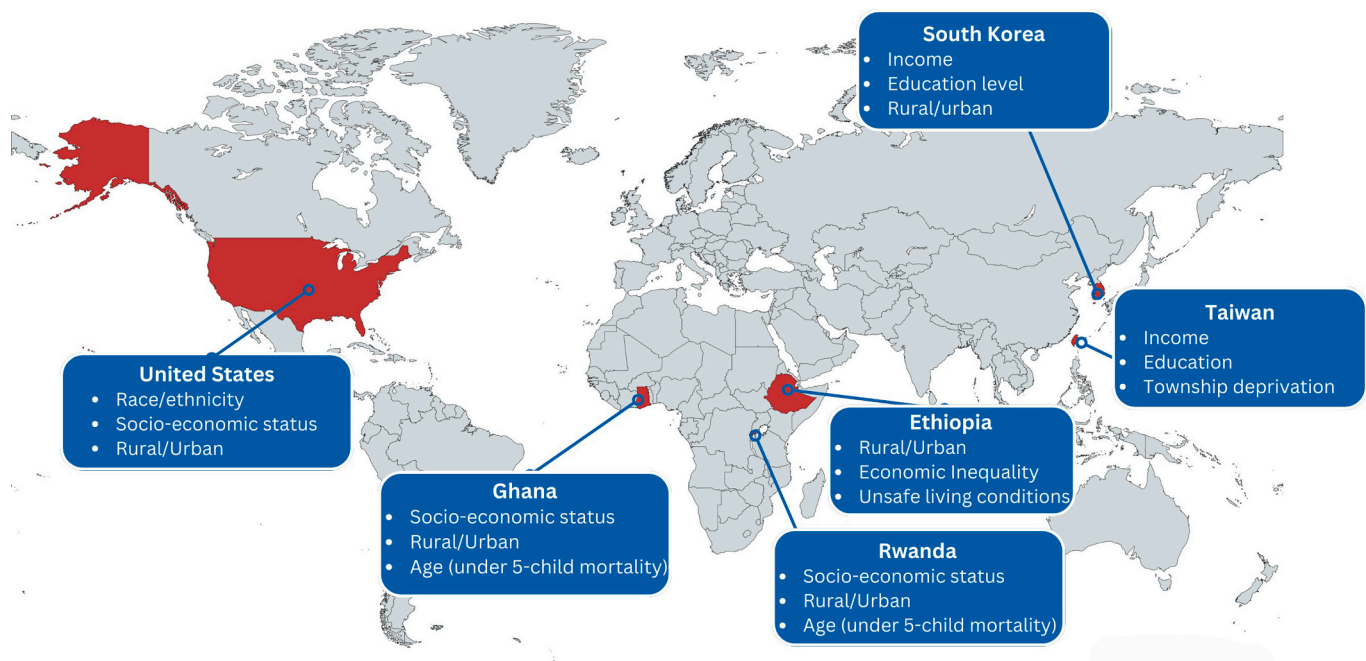


Fig. 1. Drivers of health disparities in global contexts.

characteristics (Blanchet Garneau et al., 2018). Looking ahead, we advocate for national datasets and research designs to incorporate disaggregated and intersecting sociodemographic variables. More explicitly describing the samples' multiple demographic characteristics through intersectional approach would better capture the full diversity of the populations we serve (Blanchet Garneau et al., 2018). Hence, we encourage nurse educators to critically assess dominant frameworks that may reinforce racial hierarchies or exclude non-Western epistemologies in classroom content or approaches.

3. Global perspectives on geo-political contexts and education implications

To better understand different framings of health disparities and social identifiers across settings, we draw on insights from Taiwan, Rwanda, and other conflict-affected regions. Understanding how multiple frameworks explain the intersection of global and local forces shaping health disparities can help nurse educators rethink pedagogical approaches and foster cultural humility (Foronda et al., 2016).

Erh-Chi Hsu illustrates this viewpoint by explaining how health disparity discussions in Taiwan tend to focus on gender, income, rural-urban divides, and age, with minimal attention to race or ethnicity (Weng et al., 2021). Identifying individuals by their race or ethnicity, without considering historical or national contexts, has been critiqued as a reductive and potentially harmful form of cognitive warfare (Hung and Hung, 2022). In Taiwan, race and ethnicity are not dominant demographic categories, as they can obscure individual and collective identities. Many Taiwanese actively resist the homogenization of Chinese and Taiwanese people and view it as political propaganda to influence perceptions of unification (Hung and Hung, 2022). In this context, many people recognize oppression to rise from authoritarian governance, rather than from colonization or racism. This exemplifies how the meanings and consequences of social identifiers are situated and contingent on each historical background.

Samuel Byiringiro from Rwanda offers another perspective on the social construction of race and ethnicity. Colonial regimes created three ethnic groups—Tutsi, Hutu, and Twa—which later fueled the 1994 genocide against the Tutsis by extremist members of Hutu ethnic group, resulting in over one million deaths and displacement of two million people (Mamdani, 2001). The post-genocide Rwandan government abolished ethnic categorization to promote national unity and reconciliation (Mamdani, 2001). While people may no longer recognize ethnic classifications, the long-term consequences of conflict, including psychosocial challenges, economic setbacks, and regional insecurity, affect socio-economically disadvantaged individuals, because they may struggle to get the support they need (Mamdani, 2001). Getachew Kassa further asserts that poverty, rather than ethnicity, is a primary driver of disparities in conflict-affected settings. Displacement due to wars limits access to clean water, sanitation, and healthcare, and exacerbates vulnerability to infectious diseases (Marou et al., 2024). The recent re-emergence of polio, a nearly eradicated disease, in Gaza illustrates how conflict-related conditions create critical health disparities.

Examples above demonstrate how people understand race and ethnicity differently across distinct cultural, historical, and political regions. For nurse educators, we emphasize the importance of designing curricula and pedagogical approaches that integrate relevant social determinants across different contexts, while recognizing race, though important, as one of several intersecting axes of disparity. Practical strategies to incorporate global perspectives and cultural humility include: integrating case studies from different geopolitical contexts; fostering critical dialogue on health challenges through virtual global health simulations and journal clubs; and encouraging reflexive discussions about students' own positionalities and how they shape their viewpoints (Kohl and McCutcheon, 2015; Nye and Dillard-Wright, 2023; Thistlethwaite et al., 2012; Umphrey et al., 2022). Nursing education should actively employ such approaches to move beyond a narrow focus

on race and ethnicity as contributors to health disparities and expand applicability across diverse sociocultural contexts.

4. Various axes of disparities in need of service improvement, research, and education

Examples below from Ethiopia and Ghana illustrate how systemic issues, such as infrastructure, healthcare workforce shortages, and economic constraints, prominently shape health disparities, compared to the race-based focus often seen in the US (Osei-Assibey, 2014; Woldemichael et al., 2019). Regions like Sub-Saharan Africa emphasize socioeconomic factors, such as income, geography, and infrastructure, as critical drivers of health disparities. In Ghana, for instance, socioeconomic status, particularly the intersection of income and geographic location, significantly influences health disparities, while race is less central due to relatively homogeneous social structures (Osei-Assibey, 2014).

Pedagogical approaches in this region thus prioritize optimization of limited resources and implementation of national healthcare schemes for equitable healthcare service distribution in underserved rural areas (Osei-Assibey, 2014). Kassa explains how health disparity education in Ethiopia also emphasizes local contexts and community, including the role of health extension workers who are community-based professionals trained to deliver services in underserved rural areas (Assefa et al., 2019). The focus is on enhancing access to basic services and reducing the impact of chronic and infectious diseases by building workforce and regional capacities to track and manage them (Woldemichael et al., 2019).

In contrast, South Korea's health disparities, though also shaped by urban-rural divides, are influenced by a different configuration of factors. The concentration of healthcare resources, including specialist physicians and tertiary hospitals, in metropolitan areas creates challenges for rural populations who must travel long distances for advanced care (Ju et al., 2024). This issue is compounded by aging rural population and migration of younger individuals to urban areas, which further exacerbate geographic disparities in healthcare access (Ju et al., 2024).

Additionally, gender and sexuality intersect with key drivers of health disparities—such as race, age, income, geographic location, and infrastructure in these examples—which result from the interplay of sociopolitical and systemic factors (Beckfield and Olafsdottir, 2013). Gender also structures how nursing knowledge travels and whose expertise is seen as authoritative, as labor and authority in health systems are gendered (Dunn, 2025). Considering this, we are mindful that we are a team of cis-hetero scholars and acknowledge that the health of LGBTQ+ and gender-diverse individuals is an area of health disparity globally that warrants dedicated attention. These points underscore the need for context-specific pedagogical strategies in nursing education to better prepare nurses to consider multiple axes of disparities that operate in similar and different ways across geographically varied contexts.

5. Future plans, reflections, and insights of the international scholars

We critically examine US-based research and education by drawing on both US and home-country experiences. Training in the US can broaden perspectives and foster collaboration for both local and international scholars. International scholars can also illuminate their home contexts by recontextualizing what they learned in the US and offering insights into how disparities emerge and the relevant interventions for those settings.

Importantly, we acknowledge our privileges regarding how training in the US shapes which nursing curricula are exported and centered beyond the US context. This recognition requires a deeper interrogation of the lessons we take back to our home countries and engagement with anti- and decolonial scholarship to continually assess existing

hierarchies of knowledge and power (Gondwe et al., 2024). We call for expanding the US-based health disparities framework through collaborative knowledge production. We posit that scholars and clinicians across the Global North and South, both historically better-resourced and less-resourced settings, together with community partners—including the recipients of the services, research, and education we provide—should share responsibility for developing a more globally nuanced body of knowledge on health disparities.

As the number of international scholars in the US and the broader impact of US policies on global health continue to grow, Byiringiro argues that health disparities education should encompass transnational health disparities and the influence of global health policies. Reflecting on his graduate training in global health equity in Rwanda, he emphasizes the need to redefine global health. Transnationally, global health has focused on interventions in low-income countries by high-income countries, but it should now be understood as improving health outcomes for disadvantaged populations, including those in high-income countries (Chen et al., 2020).

Soo Hyun Kim highlights the importance of addressing disadvantaged populations in a relatively homogeneous, high-income country. Her exposure to social determinants of health and intersectionality during US-based nursing scholarship has led her to integrate the intersectional framework into the Korean context. Despite the ethnic homogeneity, disadvantaged groups in South Korea, such as North Korean defectors, face disparities and experience different forms of trauma and discrimination, resulting in poor quality of life, low socio-economic status, and social isolation (Shin et al., 2021). Kim suggests that an intersectional approach to North Korean defectors' disparities can more visibly reveal the various factors shaping health disparities, leading to closely tailored care and partnerships with the community (Blanchet Garneau et al., 2018). Although the concept of intersectionality originates in Black feminist scholarship, we draw on it as an applicable analytic lens to examine the co-constituted axes of privilege and oppression in other settings, when carefully situated within local histories and power dynamics.

Sarah Won further discusses Korea's changing demographics, with an increasing number of low wage laborers from other countries and transnational marriages (Korea Statistical Information Service, 2024). This growing cultural diversity calls for proactive measures to address health disparities within these new, diverse communities. This change highlights the importance of integrating health disparities, cultural humility, and cultural safety into nursing curricula—areas that have been insufficiently applied in Korea's traditional nursing education (Park et al., 2019).

Thomas Hinneh shares how his research approach to hypertension care has been shaped by experiences from Ghana and the US. In the US, disparities in hypertension control are largely influenced by healthcare access and other dynamics viewed through the lens of social determinants of health and race, whereas in Ghana, socioeconomic divide, infrastructural, and workforce challenges are more prominent (Hinneh et al., 2024). Drawing on insights from both settings, Hinneh aims to develop a clinically oriented hypertension management program grounded in the World Health Organization's team-based care policy, tailored to address barriers and the socioeconomic realities of disadvantaged groups in each country (Hinneh et al., 2024).

These stories highlight how mutual learning and cross-cultural exchange can expand the boundaries of nursing curricula. When scholars from different backgrounds collaborate to adapt US-based frameworks to global contexts and incorporate insights from other countries into US scholarship, they can foster deeper cultural sensitivity and design more context-specific approaches to health disparities. Collaborative initiatives between international institutions, such as reciprocal learning workshops on virtual platforms (Vicente et al., 2022), can bring together culturally nuanced approaches that are translatable across settings.

Having nursing students experience cross-sector collaboration is another approach to prepare them for the complexities of applying

health disparities interventions. Community-partnered internships and policy brief workshops offer valuable opportunities to engage directly with minoritized communities and understand local and global forces (Doubeni et al., 2022; Lee and Choi, 2022). These hands-on experiences can teach students to tailor interventions to the real-world needs of diverse communities. Throughout, it is crucial to emphasize that no single intervention fits every context. Training in cultural humility and cultural safety alongside critical self-reflection on one's power and biases should come first, as these reflections enable authentic mutual learning (Gondwe et al., 2024). Curricula should prioritize critical thinking, cultural humility and safety, advocacy skills, and ongoing reflexivity to equip future nurses to design and implement effective, context-sensitive care.

6. Key recommendations for addressing health disparities

We recommend the following actionable strategies to rethink nursing research and education on health disparities:

- Addressing disparities among disadvantaged populations across all systems.
- Incorporating transnational and local sociopolitical perspectives to examine how global, national, and local health policies, migration, and geopolitical conflicts shape disparities.
- Considering multiple axes of disparities and demographic changes to better understand and address the needs of minoritized groups.
- Promoting partnerships with minoritized communities to collaboratively design culturally relevant interventions and inform evidence-based policies.

Fig. 2 shows the strategies and educational modalities to foster critical engagement and application.

7. Conclusion

We call for ongoing transformation in US-based nursing research and education scholarships to incorporate pluralistic understandings of health disparities that capture the complex interplay of social determinants across varying contexts. Practical strategies outlined include cultural humility and safety, critical self-reflection, careful application of intersectionality, and cross-national learning. Developing nursing curricula and research priorities through collaborative knowledge production among global scholars, clinicians, and community partners can better equip future nurses to address health disparities in globally informed ways. Continuous attention should be paid to whose voices are represented and which are missing throughout the process. These approaches will help strengthen nursing scholarships to play a pivotal role in reducing health disparities within the US and globally.

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Soo Hyun Kim: Writing – review & editing, Writing – original draft, Visualization, Conceptualization. **Celeste Chavez:** Writing – review & editing, Writing – original draft, Visualization, Conceptualization. **Thomas Hinneh:** Writing – review & editing, Writing – original draft. **Erh-Chi Hsu:** Writing – review & editing, Writing – original draft. **Getachew Kassa:** Writing – review & editing, Writing – original draft. **Sarah Won:** Writing – review & editing, Writing – original draft. **Samuel Byiringiro:** Writing – review & editing, Writing – original draft, Visualization.

Ethics

Ethical approval was not required as this is a contemporary issues paper not involving any data collection.



Fig. 2. Reimagining health disparities education strategies and modalities.

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The authors used ChatGPT to copyedit parts of the manuscript. After using this tool, they reviewed and edited the content as needed and take full responsibility for the published article's content.

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